

Equity as an Operating System

The only credible measure of our seriousness will be whether the last woman, in the last village, experiences progress as a lived reality

FOR GLOBAL LEADERS, equity is often discussed as an aspiration. In India, we have learnt, sometimes painfully, that equity is something far more exacting. It is a delivery problem.

It is measured not by declarations made on global stages, but by whether a woman reaches care in time, whether a frontline worker has the tools and authority to act, whether a referral works at two in the morning, and whether dignity is embedded in systems rather than appended after failure. Equity succeeds or fails at the point of execution.

This conviction is central to the Alliance for Global Good: Gender Equity and Equality, launched at Davos in 2024. The Alliance was created to close a persistent gap in global development efforts: the distance between intent and outcomes. Across health, education, and enterprise, its mandate is simple: convert proven ideas into scalable, investable models that travel across borders.

Within a year, that mandate translated into action. At Davos 2025, partnerships were formed with the Commonwealth Secretariat to expand women's entrepreneurship and sustainable economic opportunity, and with the US-India Strategic Partnership Forum to cre-



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ate pathways for women's leadership in traditional energy sectors.

Nowhere is the importance of this approach clearer than in India's maternal health journey—one that carries lessons far beyond our borders. When India's Maternal Mortality Ratio declined from 130 (2014–16) to 88 (2020–22), this was not simply statistical progress. It was evidence of systems beginning to function with greater reliability: expanded institutional deliveries, higher coverage of skilled birth attendance, and growing trust between families and the health system, as reflected in NFHS-5 data.

But progress raises the bar.

Once scale is achieved, quality becomes the true test—consistency across geographies, the strength of last-mile clinics, the speed of emergency obstetric response, the resilience of blood supply and transport systems under pressure. As I emphasised elsewhere, the last mile is only as strong as the last clinic.

This is why women's health cannot be treated as a narrow sectoral issue, confined to pregnancy and childbirth. Maternal outcomes are shaped long before labour—by nutrition, anaemia, adolescent health, mental well-being, awareness, timely antenatal care, and continuity after birth. Women's lives do not occur in silos, and neither should our